TRANSFORMING AMERICAN HEALTHCARE

Part V: The Path Forward

Chapters 16–17 — Updated April 2026, v1.1.0

PART V: THE PATH FORWARD

CHAPTER 16: Advisory Infrastructure — From Analysis to Action

The Gap Between Knowing and Doing

Every concept in this book — the six-pillar framework, the AHEAD model mechanics, the HEDIS optimization methodology, the FHIR interoperability standard, the HEROI equity metric — exists at the intersection of two different challenges. The first challenge is understanding: grasping the concept clearly enough to apply it correctly. The second challenge is doing: translating the concept into a specific decision, program, contract, or investment.

The HTR platform's intelligence tools, research lab, and academy address the first challenge. Advisory services address the second.

Advisory practice differs from platform access in a fundamental way: it is not about providing information that a sophisticated professional cannot find themselves. It is about applying specialized expertise to a specific organizational context, translating analysis into recommendations that account for the political economy, resource constraints, operational capabilities, and stakeholder dynamics of a particular health system, payer, or state agency — and then staying present through implementation to adapt those recommendations as conditions change.

The Five Advisory Service Lines

HTR's advisory practice is organized around the five analytical pillars, with each service line delivering specialized expertise in its domain while drawing on the full six-pillar framework for cross-pillar dependencies.

Service Line 1: Policy Strategy

What it delivers:

Policy Strategy engagements help health systems, payers, state agencies, and investor groups navigate regulatory change and payment reform policy. The work ranges from federal legislative impact analysis (what does this proposed rule mean for our organization?) to state waiver design (how should we structure this 1115 waiver to achieve our coverage goals while meeting federal budget neutrality requirements?) to regulatory compliance strategy.

Typical engagement structure:

Phase 1 — Landscape and Exposure Assessment (2–3 weeks):

- Map the specific regulatory changes relevant to the client's operations
- Assess financial and operational exposure under each scenario
- Identify timeline and action requirements

Phase 2 — Strategy Development (3–4 weeks):

- Develop the policy response strategy (comment letter, waiver application, compliance roadmap, or legislative advocacy)
- Model the financial impact of policy alternatives using the Research Lab Policy Simulator
- Develop stakeholder communication framework

Phase 3 — Implementation Support (ongoing):

- Monitor rulemaking developments
- Update strategy as final rules differ from proposed rules
- Support legislative testimony or comment letter drafting

Vermont application:

Vermont policymakers have engaged HTR Policy Strategy for:

- AHEAD Model CMMI negotiation support — specifically on benchmark methodology and accountability architecture
- Act 167 reference-based pricing implementation framework
- Federal lobbying strategy for Medicare FFS AHEAD participation

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Service Line 2: Value-Based Care Transformation

What it delivers:

VBC Transformation engagements guide organizations through the full arc of APM adoption — from readiness assessment through contract negotiation, clinical program design, and year-over-year performance optimization.

This is HTR's highest-volume service line by engagement count, reflecting the central role of payment reform in the transformation agenda.

The VBC Transformation Assessment Framework:

HTR's structured readiness assessment evaluates 30 dimensions across six domains:

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VBC TRANSFORMATION READINESS ASSESSMENT
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Domain 1: Strategic Clarity (5 dimensions)
  □ APM type and contract selection rationale
  □ Executive sponsor and governance structure
  □ VBC transition timeline and milestones
  □ Financial modeling and break-even analysis
  □ Board-level risk appetite assessment

Domain 2: Data and Technology (6 dimensions)
  □ Attribution list generation capability
  □ Total cost of care measurement
  □ Care gap identification and workflow
  □ Risk stratification and care tier assignment
  □ Quality measure reporting automation
  □ EHR interoperability for network data

Domain 3: Care Delivery Capability (6 dimensions)
  □ Care management program (tier 1–4 capacity)
  □ Primary care PCMH readiness
  □ Behavioral health integration
  □ Post-acute care network relationships
  □ Community health worker deployment
  □ Patient outreach and engagement capability

Domain 4: Network and Partnerships (4 dimensions)
  □ Specialist alignment and communication
  □ Post-acute care quality and cost control
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  □ Payer relationship and contract history
  □ Community partner SDOH referral network

Domain 5: Revenue Cycle (5 dimensions)
  □ HCC coding completeness
  □ Clean claim rate and denial management
  □ VBC contract payment reconciliation
  □ Attribution denial management
  □ RAF gap identification and remediation

Domain 6: Workforce Operations (4 dimensions)
  □ Care coordination staffing model
  □ Credentialing cycle time
  □ Clinical turnover and retention
  □ Training capability for new care models
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Each dimension is scored 1–4 (Pre-transition → Developing → Operational → Optimized). A total score below 60/120 indicates the organization is not ready for full-risk VBC; the gap analysis identifies the highest-priority investments before contract entry.

Service Line 3: Health Technology Assessment

What it delivers:

HTA engagements evaluate digital health solutions, clinical AI tools, and care delivery technologies for clinical effectiveness, cost-effectiveness, implementation fit, and equity implications.

The HTA Deliverable: The Evidence Dossier

For each evaluated technology, HTR produces an evidence dossier with five sections:

1. Clinical Evidence Review:

- Systematic review of published evidence (RCTs, observational studies, real-world evidence)
- Evidence quality grading (GRADE methodology: High/Moderate/Low/Very Low)
- Effect size estimates with confidence intervals
- Applicability to the client's patient population

2. Cost-Effectiveness Analysis:

- ICER calculation (using the CEA Calculator methodology)
- Threshold comparison (ICER vs. $100K/QALY, $150K/QALY, $200K/QALY)
- Budget impact model (5-year financial impact at projected utilization)
- Sensitivity analysis on key uncertain parameters

3. Implementation Fit Assessment:

- Technology readiness requirements (FHIR integration, EHR compatibility)
- Workflow integration complexity
- Staff training requirements
- Vendor financial stability and contract terms review

4. Equity Impact Analysis:

- Does the technology have differential efficacy by race/ethnicity, income, or geography?
- Does the evidence base include diverse populations?
- Are there access barriers (cost, broadband, digital literacy) that limit equity?
- HEROI calculation (equity-weighted cost-effectiveness)

5. Governance Recommendations:

- AI governance framework (if applicable)

- Performance monitoring requirements
- Patient consent and transparency requirements
- Exit strategy if the technology underperforms

Service Line 4: Clinical Quality and Safety

What it delivers:

Clinical Quality engagements support quality improvement programs, Star Rating optimization, HEDIS performance improvement, and accreditation readiness.

The HEDIS Improvement Methodology:

HTR's approach to HEDIS performance improvement follows a five-step framework:

Step 1: Performance Baseline Using the Clinical Quality Optimizer, map current performance on all relevant HEDIS measures against national percentile benchmarks. Identify measures where the organization is:

- Below the 25th percentile (critical gap — compliance risk in VBC contracts)
- At the 25th–50th percentile (improvement opportunity with high ROI)
- At the 75th–90th percentile (maintenance required; limited marginal investment)
- Above the 90th percentile (leading performance; minimal additional investment)

Step 2: Root Cause Analysis For priority measures (below 50th percentile), analyze the denominator and numerator:

- Who is eligible for the measure (denominator) and correctly identified?
- Who in the denominator received the service (numerator) and is correctly documented?
- Where in the care pathway is the gap occurring? (not offered, offered but declined, offered and completed but not documented, other)

Step 3: Intervention Design Design measure-specific interventions targeting the identified root cause:

Root Cause	Intervention
Patients not offered service	Proactive outreach (phone, patient portal, text)
Service offered but declined	Patient education, motivational interviewing training
Service delivered but undocumented	EHR workflow modification, documentation training
Service delivered by outside provider	Claims data reconciliation, fax/portal outreach for documentation
Patients lost to follow-up	Care management touchpoint, appointment facilitation

Step 4: Implementation and Monitoring Deploy interventions with weekly performance tracking during the HEDIS measurement year. Adjust outreach intensity based on weekly close rates.

Step 5: Sustainable Systems Design Embed improvements into permanent workflows rather than leaving them as annual campaign activities. A HEDIS improvement that requires a campaign every year has not been transformed into a quality system.

Service Line 5: Health Equity Strategy

What it delivers:

Health Equity engagements design and implement equity programs — from disparity baseline assessment through SDOH screening program design, community health worker deployment, and equity reporting framework development.

The Equity Assessment Methodology:

Disparity Baseline Assessment (using Health Equity Studio):

- Identify the organization's highest-priority disparity measures (magnitude × clinical significance × intervention availability)
- Stratify quality measure performance by race/ethnicity, income, and rural/urban
- Map geographic access gaps for the service area
- Score SDOH burden by zip code for attributed population

Root Cause Analysis for Disparities: Disparities can arise from different mechanisms requiring different interventions:

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DISPARITY ROOT CAUSE TAXONOMY
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Category 1: ACCESS BARRIERS
  → Physical distance, transportation, hours of operation
  → Intervention: extended hours, telehealth, mobile units,
    transportation assistance, community health workers

Category 2: SOCIAL NEEDS BARRIERS
  → Housing instability, food insecurity, language barriers
  → Intervention: SDOH screening, community resource referral,
    CHW navigation, interpretation services

Category 3: CLINICAL PRACTICE VARIATION
  → Differential treatment quality by race/ethnicity
    (documented for pain management, cardiac workup, referrals)
  → Intervention: implicit bias training, standardized clinical
    protocols, audit and feedback on provider-level disparities

Category 4: INSURANCE AND COVERAGE BARRIERS
  → Higher uninsured rates, inadequate coverage
  → Intervention: coverage navigation, ACA enrollment
    assistance, FQHC eligibility screening

Category 5: TRUST AND ENGAGEMENT BARRIERS
  → Historical mistreatment, cultural mismatch, language
  → Intervention: community partnership, culturally congruent
    workforce, patient advisory councils
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Equity Program Design: Match interventions to root causes. Common programmatic responses:

- **SDOH Screening Program:** Systematic screening using validated tools (AHC Health-Related Social Needs Screening Tool, PRAPARE), linked to community resource referral pathways
- **Community Health Worker Program:** Hiring, training, and deploying CHWs from the communities served; embedding them in primary care teams
- **Language Access Enhancement:** Hiring bilingual staff, contracting telephonic interpretation services, translating patient education materials
- **Disparity-Focused Quality Improvement:** Dedicated QI teams working on closing specific disparity gaps in priority measures

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The Engagement Model: From Intake to Deliverable

Every HTR advisory engagement follows a structured four-phase model:

Phase 1: Scoping (Weeks 1–2)

The scoping phase transforms an initial inquiry into a defined engagement with agreed scope, deliverables, timeline, and budget.

Intake process:

- Subscriber submits intake form via the Advisory Hub

- HTR advisory team reviews and responds within 2 business days
- Discovery call (45–60 minutes): understand the specific problem, organizational context, decision timeline, and data availability
- Scope memo produced: what the engagement will and will not cover, what deliverables will be produced, what the client must provide

What makes a good scope memo:

- Specific question the engagement answers (not "help us with VBC" but "assess our MSSP Enhanced track readiness and identify the top 5 operational gaps to address before Q3 contract start")
- Defined deliverables (not "recommendations" but "VBC Readiness Assessment scorecard, gap analysis, 90-day implementation roadmap, financial model")
- Explicit exclusions (what the engagement does not cover — prevents scope creep)
- Data access requirements (what data the client must provide for the analysis to be valid)

Phase 2: Assessment (Weeks 2–4)

The assessment phase is where HTR advisors do the analytical work. For most engagements, this involves:

1. Client data review:

- Financial statements (operating margin, payer mix, revenue by service line)
- Quality measure performance data (HEDIS, Stars, MIPS scores)
- Contracts (existing payer contracts, APM agreements)
- Operational data (AR days, denial rates, credentialing cycle times, turnover rates)
- Claims data (if available) — total cost of care, preventable utilization

2. Research Lab analysis: HTR advisors use the same Research Lab tools available to Professional subscribers — APM Shared Savings Calculator, Hospital Financial Scorecard, Clinical Quality Optimizer, Health Equity Studio — but apply them to the client's specific data rather than generic scenarios.

3. Benchmarking: Compare client performance to peer benchmarks using the Innovation Leaderboard and State Performance Index — identifying where the client is above-average (strengths to build on) and below-average (gaps to address).

4. Gap identification: Synthesize data analysis, benchmarking, and stakeholder conversations into a structured gap analysis. Each gap is characterized by:

- Magnitude (how significant is the gap?)
- Root cause (why does the gap exist?)
- Solvability (can it be closed in the engagement timeframe?)
- Priority (relative to the engagement's central question)

Phase 3: Analysis and Recommendations (Weeks 4–8)

The recommendation development standard:

HTR recommendations follow a structured standard:

1. **Claim:** The specific recommended action (what to do)
2. **Evidence:** The analytical basis for the claim (from the assessment)
3. **Expected outcome:** What the recommendation will produce (quantified where possible)
4. **Preconditions:** What must be true for the recommendation to work
5. **Risk/downside:** What could go wrong, and how to mitigate it
6. **Implementation:** Who does what, by when, with what resources

Recommendations that lack preconditions and risk assessment are not complete recommendations — they are wishes. An HTR recommendation that is not implementable in the client's actual context is a failure regardless of its analytical rigor.

Deliverable standards by service line:

Service Line	Primary Deliverable	Standard Format
Policy Strategy	Policy Landscape Brief + Strategy Memo	Word/PDF, 15–25 pages
VBC Transformation	Readiness Scorecard + Implementation Roadmap	Excel (scorecard) + Word (roadmap)
HTA	Evidence Dossier + Coverage Recommendation	Structured ICER/NICE format
Clinical Quality	HEDIS Gap Analysis + Improvement Plan	Excel (measures) + Word (plan)
Health Equity	Disparity Baseline Report + Equity Action Plan	Word/PDF + Excel (metrics)

Phase 4: Implementation Support (Weeks 8–12+, optional)

Some engagements end at Phase 3 — the client has the roadmap and implements independently. Others include ongoing advisory support:

Monthly check-ins: 60-minute call reviewing implementation progress, answering questions, addressing emerging obstacles

Adaptive guidance: As implementation reveals information the assessment did not anticipate (a payer contract changes mid-implementation, a key staff member departs, a new CMS rule changes the landscape), HTR advisors update recommendations to reflect new conditions

Performance tracking: For VBC and quality engagements, monitoring agreed metrics against targets and triggering course corrections if performance diverges from projections

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Implementation Toolkits: Operationalizing Analysis

Advisory subscribers have access to a library of implementation toolkits — downloadable templates and frameworks that operationalize the analytical frameworks from HTR's advisory methodology:

Toolkit	Contents	Who Uses It
APM Design Specification Template	Payment model structure, benchmark methodology, quality withhold design	Finance, strategy teams entering new VBC contracts
VBC Contract Review Checklist	65-item checklist for reviewing APM contract terms	Legal, finance, strategy for contract negotiation
1115 Waiver Application Guide	Federal narrative requirements, actuarial documentation needs, federal review criteria	State Medicaid agencies designing waiver programs
HEDIS Improvement Workbook	Measure-by-measure gap analysis, intervention design, weekly tracking template	Quality improvement teams
Health Equity Assessment Framework	Disparity analysis template, SDOH burden scoring, CHW program design criteria	Equity officers, population health teams
AI Governance Checklist	40-item checklist for evaluating clinical AI tools before deployment	CIOs, clinical informatics teams
Hospital Financial Stress-Test Model	Excel model with payer mix, rate sensitivity, and volume stress-test tabs	CFOs, financial analysts
SDOH Screening Program Design Template	Screening tool selection, referral pathway design, data collection standards	Care management, population health

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Advisory Pricing: The Value Equation

HTR advisory engagements are priced on value delivered, not hours consumed. The pricing reflects the decisions the engagement enables and the financial impact of those decisions:

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ADVISORY ENGAGEMENT PRICING GUIDE
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Engagement Type               Duration     Indicative Range
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Policy landscape brief          2 weeks     $8K–$15K
APM readiness assessment        4 weeks     $20K–$40K
Full VBC transformation         12 weeks    $75K–$150K
HTA/technology evaluation       4 weeks     $15K–$35K
Health equity baseline + plan   8 weeks     $40K–$80K
State waiver strategy           8–16 weeks  $50K–$200K
HEDIS improvement planning      6 weeks     $30K–$60K
AHEAD Model assessment          8 weeks     $60K–$120K
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All engagements include:
  → Professional platform access for client team
  → Priority AI Analyst (Claude Sonnet 4.6)
  → Research Lab full access
  → Office hours availability during engagement period
  → All deliverables in agreed formats
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The value equation is explicit in HTR's engagement model: an APM readiness assessment at $30K that identifies a $500K RAF coding gap or prevents a $200K revenue cycle error under a new VBC contract has a 10–17× ROI before the engagement ends.

Office Hours: Expert Access Between Engagements

For Professional and Advisory subscribers not currently in a formal engagement, Office Hours provide 30-minute access to pillar-specific HTR experts for specific questions.

Office Hours are particularly valuable for:

- Interpreting a new CMS rule's implications for your organization
- Getting a second opinion on a contract term before signing
- Reviewing an internal analysis for methodological soundness
- Understanding a new CMMI model before deciding whether to participate
- Preparing for a board presentation on VBC transition

Each pillar has a dedicated expert calendar:

Pillar	Expert Focus Areas
Policy	Medicaid programs, Medicare, state legislation, waiver strategy, CMMI models
Economics	VBC financial modeling, APM contract analysis, health economics, hospital finance
Technology	Digital health, FHIR, AI governance, EHR strategy, interoperability
Clinical	Quality measures (HEDIS, Stars, MIPS), care model design, hospital quality
Equity	SDOH programs, disparity analysis, community health, CHW programs
Operations	Revenue cycle, HCC coding, workforce operations, credentialing

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Key Concepts Introduced in This Chapter

> **Evidence dossier:** HTR's structured deliverable for health technology assessments, combining clinical evidence review, cost-effectiveness analysis, implementation fit assessment, equity analysis, and governance recommendations.

> **VBC Transformation Readiness Assessment:** HTR's 30-dimension assessment framework evaluating an organization's readiness to enter and succeed in value-based care contracts across strategy,

> technology, care delivery, networks, revenue cycle, and workforce operations.

> **Office Hours:** Scheduled 30-minute sessions with HTR pillar-specific experts; available to Professional and Advisory subscribers for specific questions outside of formal engagement scope.

> **Implementation toolkit:** A downloadable template or framework operationalizing an HTR advisory methodology for independent use by subscribers.

> **Scope memo:** A structured document defining an advisory engagement's questions, deliverables, exclusions, timeline, and data requirements; produced after the discovery call to prevent scope creep.

CHAPTER 17: The Future of Healthcare Transformation — 2026 and Beyond

Where the System Is Today

Before projecting where American healthcare is going, it is worth characterizing where it actually is in 2026 — not in terms of where it should be, but in terms of what has actually changed and what has not.

What has genuinely transformed:

Coverage: The ACA's coverage expansion — Medicaid expansion in 40 states, marketplace plans with subsidies, dependent coverage to age 26, guaranteed issue — has reduced the uninsured rate from 18% in 2010 to approximately 8% in 2025. This is the most significant coverage improvement in American history, affecting more than 25 million people.

Technology infrastructure: FHIR API adoption, driven by the 21st Century Cures Act and CMS mandates, has fundamentally changed the technical architecture of health data exchange. EHR adoption is near-universal. Electronic prior authorization is becoming standard. Real-time data exchange, while still incomplete, is operationally possible at scale for the first time.

Payment reform spread: Approximately 58% of commercially insured Americans are now in some form of value-based payment arrangement — up from approximately 20% in 2015. ACO programs cover more than 12 million Medicare beneficiaries. Global budget experiments in Vermont and other states have demonstrated that all-payer cost containment is technically achievable.

Quality measurement: HEDIS, Stars, and MIPS have created a standardized quality measurement infrastructure that, despite its limitations, produces more consistent and comparable clinical quality data than existed a decade ago.

What has not changed:

Administrative complexity: The $1 trillion annual administrative burden has not materially declined. Despite FHIR mandates, prior authorization reform, and electronic transaction standards, the fundamental complexity of a multi-payer administrative system has not been reduced.

Health disparities: Racial and ethnic health disparities have not meaningfully narrowed over the past decade, despite significant policy attention, equity-focused programming, and measurement infrastructure investment. The structural drivers of disparities — housing, income, food security, segregation — are outside the healthcare system and require social policy interventions that healthcare reform alone cannot deliver.

Cost growth: National health expenditure growth has moderated compared to the pre-ACA era but has not been constrained to GDP growth. The 3.5–4.5% annual cost growth trajectory, if sustained, means per-capita healthcare spending will exceed $20,000 by 2035.

Fee-for-service dominance: Despite 15 years of payment reform effort, the majority of U.S. healthcare spending remains in fee-for-service payment arrangements. VBC penetration is real but has not yet achieved the tipping point at which FFS becomes the minority mode of payment.

Five Forces Shaping the Next Decade

Force 1: The AI Transformation of Clinical Practice

Artificial intelligence will reshape clinical practice more fundamentally in the next decade than any other single technology force. The trajectory is already visible:

Diagnostic AI maturation: FDA-cleared AI diagnostic devices for radiology, pathology, ophthalmology, and cardiology are becoming standard of care in high-volume settings. As evidence accumulates, AI-assisted diagnosis will move from optional to expected in most clinical specialties.

Ambient clinical intelligence: The next frontier in clinical AI is not image analysis but ambient intelligence — AI systems that listen to clinical encounters, generate structured documentation automatically, extract quality measure data, identify care gaps in real time, and reduce the documentation burden that drives physician burnout. Companies including Nuance (Microsoft), Ambience Healthcare, and Suki are deploying these systems at scale.

Predictive population health at the individual level: The convergence of FHIR-based data access, large language models, and electronic claims data will enable the next generation of risk stratification — not just identifying high-risk patients but predicting the specific clinical events (hospitalization, medication nonadherence, crisis escalation) that care management programs should prevent.

AI governance as a clinical requirement: As AI becomes embedded in clinical practice, AI governance — bias testing, performance monitoring, explainability requirements, and accountability frameworks — will transition from a quality initiative to a regulatory requirement. FDA, ONC, and CMS are all developing AI governance frameworks. Organizations without established AI governance infrastructure will face compliance exposure.

Force 2: The Demographic Reckoning

The United States is aging — and the healthcare system is not ready.

The Baby Boom generation is fully in Medicare (age 65+) by 2030. The oldest Boomers turn 84 in 2030. The fastest-growing age cohort in the U.S. over the next decade is adults 80 and older — the highest-cost, highest-complexity segment of the healthcare population, with the highest rates of dementia, frailty, multimorbidity, and functional limitation.

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U.S. DEMOGRAPHIC TRANSFORMATION — HEALTHCARE IMPLICATIONS
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                    2024        2030        2040
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Adults 65+         58M (17%)   73M (22%)   82M (24%)
Adults 80+         13M (4%)    17M (5%)    24M (7%)
Working-age (18–64) 188M (56%) 186M (56%) 181M (52%)

Medicare enrollment 66M        73M         80M+
Medicare spending   $1.0T      $1.5T (est.) $2.2T (est.)
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Implication: The worker-to-beneficiary ratio for Medicare
financing declines from 3.2:1 today to approximately 2.6:1
by 2035 — requiring either increased payroll taxes, reduced
Medicare benefits, or dramatic payment rate reductions to
maintain program solvency.
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Vermont is the canary in this particular coalmine: its 65+ population is rising 57% by 2040 while its working-age population declines 13%. Every challenge Vermont faces today — hospital financial unsustainability, workforce shortages, premium unaffordability, global budget stress — will be the national condition by 2035.

Implications for transformation:

- Long-term care capacity (nursing homes, assisted living, home and community-based services) must expand dramatically — and the workforce to staff it does not yet exist

- Medicare Advantage enrollment will continue growing as FFS Medicare becomes less financially viable for CMS
- Dementia care — affecting an estimated 14 million Americans by 2060 — requires new care models (dementia-specialized primary care, PACE programs, family caregiver support) that do not yet exist at adequate scale
- The AHEAD Model's Medicare FFS inclusion challenge will become more acute nationally as the Medicare population grows

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Force 3: The Consolidation of Health Systems

American healthcare is consolidating. Hospital mergers and acquisitions, health system vertical integration (hospitals acquiring physician practices, post-acute facilities, and insurers), and private equity investment have all accelerated consolidation of provider markets.

The consolidation data:

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HEALTHCARE MARKET CONSOLIDATION TRENDS
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Hospital mergers completed (annual average):
  2010–2015:    80–90 per year
  2016–2020:    110–125 per year
  2021–2025:    90–105 per year (slight moderation after
                federal antitrust scrutiny increase)

Percentage of physicians employed by hospitals or
  health systems:
  2012:   26%
  2018:   44%
  2024:   ~65%

Health system vertical integration:
  ~40% of major health systems now have an affiliated
  or owned insurance product (narrow network plan,
  MA plan, or self-funded employer product)
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Why consolidation matters for transformation:

Large, consolidated health systems have the scale to invest in VBC infrastructure — care management programs, analytics platforms, care coordination networks — that smaller, independent organizations cannot afford. This is an argument for consolidation.

But consolidation also reduces competition, and in highly concentrated markets, providers use market power to negotiate above-cost commercial rates — increasing premiums and reducing the pressure to improve efficiency. The research consistently shows that hospital mergers in already-concentrated markets increase prices without improving quality.

The policy challenge is distinguishing between consolidation that enables transformation (scale-enabling clinical integration) and consolidation that undermines transformation (market-power-driven price extraction). Current federal antitrust enforcement has focused primarily on preventing further consolidation in already-concentrated markets but has not disaggregated existing consolidated systems.

Force 4: The Drug Pricing Reckoning

Pharmaceutical spending — $722 billion in 2023 — is the fastest-growing component of healthcare spending in percentage terms, driven by:

- New biologic and gene therapies with transformative efficacy but extraordinary prices ($1–3 million per treatment)

- GLP-1 agonists (Ozempic, Wegovy, Mounjaro) with proven obesity and cardiovascular efficacy and prices that would add $250–400 billion annually to U.S. drug spending at current pricing if fully utilized
- Specialty drug category growth across oncology, immunology, and rare diseases

The Inflation Reduction Act:

The IRA's Medicare drug price negotiation provisions — allowing CMS to negotiate directly with manufacturers for the highest-spending Part D drugs — represent the first meaningful constraint on pharmaceutical pricing in the U.S. program history. The first 10 drugs were announced for negotiation in 2023 (effective 2026), with expanded negotiation cohorts in subsequent years.

Projected savings: CBO estimated $25 billion in federal spending reductions over 10 years from the initial negotiation provisions — meaningful but modest relative to the $7 trillion projected Medicare drug spending over that period.

The equity dimension of drug pricing:

High drug prices create specific equity burdens:

- Cost-related medication nonadherence (skipping doses to afford other expenses) is documented at rates of 25–30% among low-income adults for high-cost medications
- The racial wealth gap means high out-of-pocket drug costs disproportionately harm Black, Hispanic, and low-income patients
- GLP-1 agonists — potentially transformative for obesity, cardiovascular disease, and type 2 diabetes — are currently not affordable for most low-income patients, which means their benefits will disproportionately accrue to higher-income patients who can afford them

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Force 5: The Policy Pendulum

Healthcare policy has always been politically contested, and the next decade will not be an exception. The ACA's coverage architecture has survived repeated legal and legislative challenges. CMMI models have continued operating across administrations. But the policy environment will continue to create both transformation opportunities and transformation risks:

Transformation opportunities:

- Medicare for All or a public option could expand coverage and simplify administration — though political viability remains uncertain
- Mandatory APM participation (moving from voluntary to required VBC) would dramatically accelerate payment reform penetration
- A universal patient identifier (prohibited since 1998) would solve the patient matching problem that undermines FHIR interoperability
- A national SDOH investment fund — linking healthcare savings from prevention programs to community-based social services — would address the upstream drivers of high-cost utilization

Transformation risks:

- Medicaid contraction (work requirements, reduced federal matching) would reverse coverage gains among the lowest-income populations
- Telehealth restriction restoration (geographic and originating site requirements) would reduce rural access gains
- Private equity deregulation would accelerate the access and quality harms documented in PE-owned healthcare settings
- AI-related privacy erosion — without adequate HIPAA updates for AI applications — could undermine patient trust in data sharing

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The Six-Pillar Forecast: 2026–2035

Based on the five forces described above and the current trajectory of each pillar, HTR's 10-year forecast:

Policy Pillar: Moderate Progress

The policy environment will continue to produce incremental payment reform — expanding APM penetration, extending telehealth coverage, strengthening prior authorization requirements. A major structural reform (Medicare for All, national global budget) remains unlikely within the 10-year horizon due to political constraints. The most consequential policy change would be mandatory Medicare APM participation — which HTR assesses as having a 25–35% probability of adoption by 2035.

Vermont-specific: Federal approval of Medicare FFS participation in AHEAD is the highest-probability major policy change for Vermont — HTR assesses 50–60% probability by 2028 if Vermont's federal lobbying effort is sustained.

Economics Pillar: Continued Progress with Acceleration Risk

VBC penetration will continue growing, reaching 70–75% of commercially insured by 2030. ACO REACH will mature as a full-risk model. The economics disruption risk is the GLP-1 agonist spending wave — if Medicare/Medicaid coverage of GLP-1s for obesity is expanded (highly likely by 2027–2028), the drug spending impact on health system operating margins could be significant.

Global budget expansion beyond Vermont is plausible: Maryland's All-Payer Model remains the second example, and Making Care Primary's 8-state cohort may generate policy pressure for broader adoption.

Technology Pillar: Rapid Progress

Technology will be the fastest-improving pillar. FHIR interoperability will become operationally complete (not just mandated) for most major EHR systems by 2028. AI deployment in clinical settings will accelerate. The risk is AI governance lagging AI deployment — creating patient safety and equity harms before governance frameworks catch up.

Patient matching will improve through the CommonWell/Carequality networks and probabilistic matching advances, even without a universal patient identifier.

Clinical Pillar: Steady Progress

Clinical quality improvement will continue at its historical rate of gradual improvement. The behavioral health integration expansion — driven by CoCM reimbursement, Act 167-style state investments, and ACO care management requirements — will produce measurable improvements in access and outcomes for mental health conditions.

The clinical workforce challenge will intensify before it improves: the primary care physician shortage will peak around 2030–2032 before APRN scope-of-practice expansion and new residency investment begin to close the gap.

Equity Pillar: High Risk of Stagnation

Equity is the most at-risk pillar. Racial health disparities have not narrowed in the past decade; they will not narrow without structural interventions in SDOH that are outside the healthcare system's control. The most consequential equity-advancing policy — increased federal investment in affordable housing, food security, and income support — is not a healthcare policy. It requires cross-sector coordination that healthcare policymakers cannot deliver alone.

The most optimistic equity scenario: the GLP-1 wave reduces the metabolic disease burden (diabetes, cardiovascular disease, obesity) that underlies many racial and rural disparities — if the drugs can be made affordable for low-income populations, which requires either pricing reform or Medicaid/Medicare formulary decisions.

Operations Pillar: Transformation Underway

Administrative simplification — driven by ePA, FHIR-based revenue cycle automation, and AI-powered coding and billing tools — will measurably reduce administrative burden by 2028. The most ambitious scenario — a true standard national credentialing database and uniform prior authorization criteria — would reduce administrative costs by an estimated $80–120 billion annually. This would require federal coordination that is politically achievable but organizationally complex.

Vermont's 10-Year Trajectory: The National Preview

Vermont will serve as the national preview for the healthcare transformation challenges of the 2030s. By 2035, Vermont will have:

- A 65+ population that is 30% of the total population (up from ~20% today)
- A healthcare system that has either succeeded or failed at bending its cost curve under AHEAD
- A hospital sector that has either stabilized through RBP implementation and global budget success, or contracted through closures and REH conversions
- A behavioral health system that has either integrated meaningfully into primary care, or continued to generate high-cost crisis utilization

The nation's largest states — California, Texas, Florida, New York — will face Vermont's demographic and fiscal challenges on a 10–15 year lag. What Vermont proves is possible (or impossible) in the next 5 years will shape the policy toolkit available to those states as their transformations intensify.

This is the deepest reason to take Vermont seriously: not because Vermont is important in terms of population, but because it is a controlled experiment in the conditions that the entire nation is moving toward.

A Closing Framework: The Decisions That Will Determine Outcomes

Healthcare transformation does not happen automatically. It happens because specific decision-makers — legislators, regulators, hospital executives, health plan CEOs, clinicians, patients, and community members — make specific decisions at specific moments. The analytical framework this book provides is ultimately in service of better decisions.

The decisions that will most determine American healthcare's trajectory over the next decade:

Federal decisions:

1. Will CMS mandate Medicare APM participation, or continue with voluntary models?
2. Will Congress make telehealth flexibilities permanent, or restore geographic restrictions?
3. Will ONC and CMS adopt an AI governance standard, or leave governance to individual organizations?
4. Will the IRA drug pricing provisions be expanded, contracted, or left as is?

State decisions:

1. Will the remaining 11 states adopt ACA Medicaid expansion, closing the coverage gap?
2. Will additional states adopt global budget models following Vermont's path?
3. Will states strengthen or weaken prior authorization reform enforcement?
4. Will states fund SDOH investment mechanisms, or continue to leave this to healthcare organizations alone?

Organizational decisions:

1. Will health systems enter meaningful two-sided risk contracts, or remain in the shallow end of shared savings?
2. Will payers invest in genuine SDOH programs, or use "social determinants" as marketing language without operational investment?
3. Will technology vendors build truly interoperable systems, or continue competing on data lock-in?
4. Will clinical organizations deploy AI with governance rigor, or rush deployment to capture efficiency gains without adequate bias testing?

Vermont-specific decisions:

1. Will the legislature pass a viable reference-based pricing glide path in 2026?
2. Will Vermont's federal delegation succeed in negotiating Medicare FFS inclusion in AHEAD?

3. Will the behavioral health integration program be implemented with Collaborative Care Model fidelity, or diluted by implementation shortcuts?
4. Will VHCURES receive the funding and mandate it needs to support genuine global budget management?

None of these decisions is inevitable. Each one will be contested, shaped by evidence and advocacy, and ultimately made by human beings using frameworks — analytical, ethical, political — to navigate genuine complexity.

This book provides one of those frameworks. The decisions, and their consequences, belong to the people who make them.

Key Concepts Introduced in This Chapter

> **Ambient clinical intelligence:** AI systems that listen to clinical encounters, generate structured documentation, extract quality data, and identify care gaps in real time, reducing clinical documentation burden.

> **GLP-1 agonists:** A class of medications (semaglutide, tirzepatide, etc.) with proven efficacy for obesity, type 2 diabetes, and cardiovascular disease; a major cost and equity challenge for healthcare systems and payers.

> **PACE (Program of All-Inclusive Care for the Elderly):** A comprehensive care model for frail elderly patients integrating medical, social, and long-term care services; particularly relevant to the aging population challenge.

> **Making Care Primary (MCP):** CMMI's multi-payer primary care transformation model launched in 2024 with an initial 8-state cohort; a potential pathway for broader APM adoption.

> **EVPI:** Expected Value of Perfect Information; the maximum value of further research before making a decision under uncertainty; relevant to major healthcare policy decisions with uncertain outcomes.

> **Integrated delivery network (IDN):** A health system that owns and operates multiple care delivery settings (hospitals, physician groups, post-acute facilities) under a unified organizational structure; dominant form of health system organization in 2026.

End of Part V

AFTERWORD: The Platform Behind the Book

This book draws on the full analytical infrastructure of Health Transformation Review — its intelligence platform, research tools, advisory practice, and academy curriculum. For readers who want to go beyond the frameworks described here and apply them to real decisions, the HTR platform provides:

Intelligence: A continuously updated intelligence feed covering all six pillars, organized by domain and tagged for searchability. Every policy analysis, economics review, technology assessment, and equity research piece is indexed by the AI Analyst.

Research Tools: The 19 interactive Research Lab tools described in Chapter 13 — available to Subscriber, Professional, and Advisory subscribers. Apply the frameworks in this book to your specific organization, market, or state.

Learning: The HTR Academy's six learning tracks, 200+ glossary terms, case studies, and personalized learning curriculum — for systematic competency development across all six pillars.

AI Analyst: The RAG-powered AI Analyst described in Chapter 15 — available to all subscribers, with role-appropriate model routing for depth of response.

Advisory Services: For organizations that need expert guidance translating analysis into action — HTR's advisory practice described in Chapter 16.

Platform access: healthtransformationreview.com

End of Book
